

FROM POLICY TO ACTION

Strengthening Adolescent Sexual and
Reproductive Health Services in the County



EXECUTIVE SUMMARY

Adolescents and young people make up a significant proportion of Kenya's population and represent the country's future social and economic potential. The National Adolescent Sexual and Reproductive Health Policy provides a framework to ensure that adolescents have access to accurate information, quality health services and supportive environments that enable them to make informed decisions about their health and well-being.

Despite the existence of this national policy, implementation at the county level remains limited. Many adolescents continue to experience unintended pregnancies, limited access to youth-friendly health services, harmful cultural practices, stigma and inadequate access to age-appropriate sexuality education.

Counties have an opportunity to strengthen implementation through targeted investments, multi-sectoral collaboration and meaningful youth participation.

WHY THIS MATTERS

Investing in adolescent sexual and reproductive health helps to:

- ✓ Reduce adolescent pregnancies
- ✓ Keep girls in school
- ✓ Reduce maternal deaths among adolescents
- ✓ Prevent HIV and other sexually transmitted infections
- ✓ Delay early marriages and harmful practices
- ✓ Improve mental health and well-being
- ✓ Build healthier, more productive communities



CURRENT IMPLEMENTATION CHALLENGES AT COUNTY LEVEL

CHALLENGE	WHY IT MATTERS
Limited youth-friendly health services	Many adolescents avoid seeking care because services are not adolescent-sensitive or accessible in their area.
Limited implementation of comprehensive sexuality education (CSE)	Many adolescents lack accurate, age-appropriate information to make informed decisions
Cultural and religious barriers	Stigma, myths and social norms discourage adolescents from seeking information and services
Limited youth participation	Adolescents are rarely involved in designing, implementing or evaluating programmes that affect them
Weak county coordination	Limited collaboration between the health, education, youth and social sectors affects service delivery.



PRIORITY PROVISIONS AND COUNTY RESPONSIBILITIES

POLICY PROVISION	WHAT THE POLICY PROVIDES FOR	COUNTY KEY RESPONSIBILITY	COUNTY ACCOUNTABILITY
1. Youth-Friendly Health Services	Establish accessible, affordable, confidential and respectful services for adolescents including counselling and clinical services	County Department of Health leads. County Health Management Team (CHMT) to strengthen health services	Establish youth-friendly service delivery points; train health workers; monitor service quality and confidentiality; provide commodities and supplies; and track utilisation rates disaggregated by adolescents within facilities
2. Prevention of Adolescent Pregnancy	Reduce adolescent pregnancy through access to information, counselling, reproductive health commodities, life skills education and community engagement	County Departments of Health, Education and Youth Coordination; Social Services lead joint prevention programmes	Expand access to adolescent SRHR information and services; strengthen family planning; engage parents, guardians, teachers and community leaders; and strengthen prevention by addressing underlying causes early pregnancies
3. Comprehensive Sexuality Education (CSE)	Promote age-appropriate, evidence-based CSE using a rights-based and culturally sensitive approach	County Department of Education in collaboration with Department of Health, Teachers Service Board (TSB)	Support implementation of national school health and life-skills curriculum; provide teaching materials; build teacher capacity; and ensure inclusion of out-of-school youth and adolescents with disabilities in CSE
4. HIV and STI Prevention	Improve adolescent access to HIV, STI prevention, counselling, testing, treatment and behavioural services	County Department of Health leads in partnership with communities	Integrate adolescent-friendly HIV services including testing, treatment, PrEP services and linkage to care; raise awareness and reduce stigma
5. Prevention of Harmful Practices	Protect adolescents from child marriage, FGM, sexual exploitation and violence; and promote other harmful practices	County Departments of Health, Gender and Children, Youth Affairs, Law Enforcement and Community Leaders	Strengthen prevention programmes; establish referral pathways for survivors; work with community leaders to transform norms that support harmful practices and address harmful practices in schools and families



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HEALTHIER FAMILIES. STRONGER COMMUNITIES.

County plans today.
Healthy communities tomorrow.



COUNTY PRIORITY ACTIONS AND RESPONSIBILITIES

PRIORITY AREA	WHAT THIS MEANS	COUNTY LEAD	KEY ACTIONS
6. Adolescent Participation	Ensure adolescents participate in planning, implementation, monitoring and advocacy on programmes affecting them.	County Government Youth Department CHMT, CSOs	- Establish youth advisory forums; involve adolescents in county planning and public participation. - Facilitate regular youth stakeholder reviews including working groups.
7. Power and Community Engagement	Promote youth-led advocacy and engage communities, media and cultural leaders to support adolescent health.	Community Health Services, Community Health Promoters (CPHs), Religious Leaders, Cultural Leaders, Civil Society	Conduct community dialogues; strengthen community champions; engage faith and cultural leaders; use media, arts and local events to raise awareness of adolescent SRHR.
8. School-Based Interventions	Strengthen school health programmes; provide students and teachers with regular and parenting learners.	County Department of Education; School Administrators; Teachers; County Health Department	Strengthen school health facility referral systems; support implementation of school health programmes; facilitate quarterly and sessions of guidance and parenting learners in accordance with national policy.
9. Capacity Building	Build the capacity of healthcare workers, teachers, CPHs and peer educators to provide adolescent-responsive services.	County Department of Health; CHMT; Department of Education; CSOs; Training Institutions	Develop county training plans; integrate adolescent SRHR in pre-service, professional development (CPD) programmes; supportive supervision; train peer educators and Community Health Promoters.
10. Multi-sectoral Coordination	Promote collaboration among health, education, youth, gender, civil society, faith-based organizations and development partners.	County Executive Committee Member, Health Department; County Education Department; Gender and Social Services, CSOs; Development Partners	Establish or strengthen county multi-sectoral coordination structures; schedule regular workplans; coordinate implementation and resource mobilization across sectors.
11. Monitoring and Evaluation	Strengthen data collection, monitoring, research and use of evidence to improve adolescent SRHR programmes.	County Department of Health; Health Records and Information Officer; CSOs; County Monitoring & Evaluation Unit	Collect and analyze adolescent SRHR data; develop county indicators; monitor implementation; publish progress reports; use evidence to inform planning, budgeting and programme improvements.

COUNTY IMPLEMENTATION ROADMAP

PRIORITY ACTION	LEAD INSTITUTION	TIMELINE
1. Establish youth-friendly service points	County Department of Health	0–12 months
2. Train health workers on adolescent-friendly services	CHMT	Continuous
3. Strengthen collaboration with the education sector	Department of Health & Education	6–12 months
4. Conduct community awareness dialogue	CHPs, CSOs, religious and community leaders	Continuous
5. Establish youth participation forums	County Government & Youth Department	Annual
6. Develop monitoring indicators and annual reviews	County Department of Health	Annual

COUNTY OVERSIGHT CHECKLIST

QUESTION	YES	NO
1. Are youth-friendly services available in county health facilities?	☐	☐
2. Have health workers been trained in adolescent-friendly service delivery?	☐	☐
3. Are adolescents involved in county planning and decision-making?	☐	☐
4. Are schools and communities receiving age-appropriate SRHR information?	☐	☐
5. Are county budgets supporting adolescent health programmes?	☐	☐
6. Are annual reports on adolescent health being produced?	☐	☐



County governments, schools, health workers, parents, faith leaders and young people all have a role to play in ensuring that adolescents have access to the information, support and services they need to grow into healthy, empowered adults.